

Case Number :

Collection Date :

Surgical Pathology Report

Diagnosis:

A: Lymph nodes, right pelvic, regional node dissection
- No tumor seen in six lymph nodes (0/6).

B: Lymph nodes, left pelvic, regional node dissection
- Metastatic high grade urothelial carcinoma, involving 4 of 5 lymph nodes, size of largest lymph node metastasis 1.4 cm, no definite extranodal extension of tumor identified, tumor is present in afferent lymphatics in several lymph nodes.
- Immunohistochemistry confirms that the tumor in the lymph nodes is urothelial carcinoma. It is CK7 and p63 positive, PSA and PAP negative. Blocks B2 and B 3 were stained.

C: Bladder and prostate, cystoprostatectomy

Bladder

Tumor histologic type: invasive urothelial carcinoma. Immunohistochemistry confirms that the tumor in the bladder is urothelial carcinoma. It is CK7 and p63 positive, PSA and PAP negative.

Tumor histology grade (WHO classification): high grade

Microscopic tumor size (greatest dimension): 3.2 cm

Microscopic extent of invasion: into muscularis propria

Focality of tumor: unifocal, tumor is present in the anterior wall of the bladder

Angiolymphatic space invasion: present, extensive

Presence/absence of intraepithelial abnormality: A minimal amount of urothelial carcinoma in situ is present in urothelium immediately adjacent to invasive carcinoma. Sections of the bladder away from the main tumor mass are free of intraepithelial abnormalities.

Ureters: no tumor or carcinoma in situ identified, bilaterally

Histologic assessment of surgical margins:

Ureter, left: free of tumor

Ureter, right: free of tumor

Urethra: free of tumor

Paravesicular soft tissue: free of tumor

Other significant findings: A focal area of papillary cystitis is present in the dome of the bladder (the suspicious lesion seen grossly is the dome of the bladder shows only papillary cystitis with no neoplasia identified).

Lymph nodes: see specimens A and B, above. Total node count is metastatic urothelial carcinoma in 4 of 11 lymph nodes

AJCC staging (bladder):

pT2

pN2

Prostate:

Tumor histologic type: prostate adenocarcinoma. Immunohistochemistry confirms that the tumor in the prostate is prostate cancer. It is PSA and PAP positive, CK7 and p63 negative.

Tumor grade (Gleason system): 8 (4+4)

ICD-0-3

Carcinoma, Urothelial NOS 8120/3

Site: bladder, NOS C67.9

2/10/11
W



Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HPV Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle): QUALIFIED / DISQUALIFIED		
Reviewed (signature)	KMI	H2011

Approximate percentage of prostate involved by tumor: 10%, greatest tumor dimension approximately 3 cm

Extent of tumor:

Location of tumor involvement in prostate: Right and left posterior lobes, from mid to base

Confinement/non-confinement of tumor within the prostatic capsule: Tumor not confined to prostate capsule

Location of extracapsular tumor involvement: Right and left seminal vesicle

Type of extracapsular tissue involved within tumor: seminal vesicle

Angiolymphatic space invasion: Not identified

Seminal vesicles: Muscle wall of both right and left seminal vesicles involved with prostate adenocarcinoma. Adventitial tissue around seminal vesicles involved with prostate adenocarcinoma. The tumor in the seminal vesicles is confirmed to be prostate cancer. It is PSA and PAP positive, CK7 and p63 negative.

Histologic assessment of surgical margins: Surgical margins free of tumor

Other significant findings: Benign prostatic hyperplasia

Lymph nodes: See specimens A and B above. There is no evidence of metastatic prostate cancer in any lymph node. The tumor in the nodes stains as urothelial carcinoma, and is PSA and PAP negative.

AJCC Staging (prostate):

pT3b

pN0

This staging information is based on this pathologic specimen and may be incomplete. A comprehensive review of all available information is recommended to determine final staging.

Clinical History:

a clinical diagnosis of bladder CA, undergoes cystoprostatectomy.

Gross Description:

Received are three appropriately labeled containers.

Container A is additionally labeled "right pelvic lymph node" and is a 5 x 4 x 2 cm aggregate of multiple fragments of yellow/brown firm lobulated fibroadipose tissue. The fragments are palpated for lymph node candidates. Multiple lymph node candidates from approximately 5 x 4 x 3 mm to 2.5 x 1.3 x 1.0 cm (disrupted) are identified.

Block Summary:

A1 - Two lymph node candidates

A2 - One lymph node candidate, sectioned

A3 - One lymph node candidate, sectioned

A4 - One lymph node candidate, sectioned (suspicious)

A5-A7 - Largest disrupted lymph node candidate, serially sectioned

Some fibrofatty tissue is retained in formalin.

Container B is additionally labeled "left pelvic lymph node" and is a 6 x 4 x 3 cm aggregate of multiple fragments of focally roughened brown/yellow fibrofatty tissue. Many of the fragments are disrupted. Many of the lymph node candidates are very firm. A portion of one of the mid sized (2.4 cm in greatest dimension) firm lymph node candidates was given to tissue procurement.

Five lymph node candidates from approximately 6 x 6 x 7 mm to approximately 4.8 x 1.6 x 1.0 cm are identified. The second largest lymph node is received markedly disrupted but it is treated as a single lymph node. The largest lymph node candidate is likely fatty replaced.

Block Summary:

B1 - One lymph node candidate, sectioned

B2 - One lymph node candidate, sectioned (suspicious)

B3 - Two sections from firm suspicious lymph node (portion given to tissue procurement)

B4 - Two sections from large firm disrupted lymph node candidate

B5-B9 - Largest lymph node candidate, serially sectioned

Some fibrofatty tissue as well as remaining suspicious lymph nodes are retained in formalin.
Specimen C:

Specimen fixation: Formalin

Type of specimen(s) received: Unspecified cystoprostatectomy

Size of urinary bladder: The entire specimen is 18 x 9.5 x 3 cm; the bladder is approximately 8 x 8 x 3 cm and the attached prostate is approximately 6 x 6 x 5 cm.

Orientation: None provided. Inking: Anterior/blue, posterior/black, and urethral margin/yellow.

Tumor size: 3.2 x 1.2 x 0.5 cm; 2.2 x 1.2 x 0.3 cm.

Tumor location: There are two suspicious areas identified. The largest lesion is located on the anterior wall and the smaller lesion is located at the dome. The two lesions are 2 cm from one another. The anterior wall lesion is firm, red/white with some gray/yellow discoloration. The lesion at the dome is pink/tan and papillary appearing.

Other mucosal lesions: The posterior wall is erythematous, as is the trigone. The bladder was received with a Foley in place.

Gross description of other organs or structures: The right seminal vesicle is approximately 1.8 x 1.8 x 1.2 cm, the left seminal vesicle is approximately 1.7 x 1.5 x 1.0 cm. The right seminal vesicle upon cross sectioning has a firmer whiter cut surface than the left. Sectioning the prostate exhibits tan spongy prostatic parenchyma in the posterior lobe. The anterior lobe (and much of the prostate) is distorted by marked periurethral nodularity. In the right posterior lobe, there are two separate well-circumscribed white nodules from 6 x 3 x 3 mm to approximately 1.4 x 1.2 x 0.7 cm.

Digital photograph taken: No

Tissue submitted for special investigation: Tumor from anterior wall lesion and normal bladder was given to tissue procurement.

Block Summary:

- C1 - Urethral margin, en face
- C2 - Right ureteral margin, en face
- C3 - Left ureteral margin, en face
- C4 - Anterior wall lesion with blue inked anterior margin
- C5 - Representative anterior wall lesion
- C6-C7 - Full thickness anterior wall lesion, slice bisected, C7 has unlinked peritoneum
- C8 - Normal bladder between anterior and dome lesion
- C9-C13 - Dome lesion, serially sectioned and entirely submitted
- C14 - Normal posterior wall
- C15 - Right ureteral orifice
- C16 - Left ureteral orifice (internal)
- C17 - Additional left lateral wall
- C18 - Trigone
- C19 - Right seminal vesicle
- C20 - Left seminal vesicle
- C21 - Representative right approximately mid prostate
- C22 - Representative approximately mid left prostate
- C23 - Smaller right posterior lobe well circumscribed right nodule
- C24-C27 - Larger right posterior lobe nodule, serially sectioned and entirely submitted
- C28 - Contiguous section of right and left seminal vesicles (left has a small 2 mm in greatest dimension white nodule)
- C29 - Anterior lobe of prostate, towards base, trigone of bladder present
- C30-C48 - Additional posterior lobe of prostate from mid prostate towards base

Light Microscopy:

Light microscopic examination is performed

Appropriate internal and/or external positive and negative controls have been evaluated. Some of the immunohistochemical reagents used in this case may be classified as analyte specific reagents (ASR). These were developed and have performance characteristics determined by the

. These reagents have not been cleared or approved by the US Food and Drug Administration (FDA). The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high complexity clinical laboratory testings.

Signature

I have personally conducted the evaluation of the above specimens and have rendered the above diagnosis(es).